

SUPREME COURT OF THE STATE OF NEW YORK
COUNTY OF NEW YORK

-----X
JUSTIN SAPINI,

Index No. 160000/2024

Plaintiff,

-against-

**EXPERT WITNESS
EXCHANGE PURSUANT
TO CPLR 3101(d)**

ANTHONY F. FERRARA and METRO-NORTH COMMUTER
RAILROAD,

Defendants.
-----X

S I R S:

Pursuant to CPLR § 3121, CPLR §3101(d) and 22 NYCRR §202.17, the report of Dr. Emmanuel Hostin, dated March 31, 2026, concerning the independent orthopedic examination of plaintiff, JUSTIN SAPINI, is annexed hereto and herewith served upon you.

PLEASE TAKE NOTICE, that responding defendants may rely upon the expert testimony of Dr. Hostin during the trial of this action. If called, Dr. Hostin's testimony will be with regard to his findings, conclusions and opinions in accordance with his annexed report dated March 31, 2026.

PLEASE TAKE FURTHER NOTICE, that responding defendants reserve the right to amend/supplement this response, herein if any additional information becomes known/available.

Dated: Woodbury, New York

April 9, 2026

Yours, etc.

MILBER, MAKRIS, PLOUSADIS
& SEIDEN, LLP.

A handwritten signature in black ink, appearing to read "Stephen F. Zaklukiewicz", followed by a horizontal line.

By: STEPHEN F. ZAKLUKIEWICZ, ESQ.
Attorneys for Defendants
ANTHONY F. FERRARA and METRO-
NORTH COMMUTER RAILROAD
1000 Woodbury Road, Suite 402
Woodbury, New York 11797
(516) 712-4000
File No.: 1805CS-28142M

TO:

HOWARD B. STOLZENBERG
STOLZENBERG CORTELLI, LLP
Attorneys for Plaintiff
JUSTIN SAPINI
305 Old Tarrytown Road
White Plains, New York 10603
(914) 361-4888

AFFIRMATION OF SERVICE

STATE OF NEW YORK)
) ss.:
COUNTY OF SUFFOLK)

LYNN SAGL, being duly sworn, deposes and says, that deponent is not a party to the action, is over 18 years of age and resides in Suffolk County, State of New York.

That on the 9th day of April 2026 deponent served the within

EXCHANGE OF IME REPORT

upon the following attorney(s) for that purpose by **E-FILING with NYSCEF**.

HOWARD B. STOLZENBERG
STOLZENBERG CORTELLI, LLP
Attorneys for Plaintiff
JUSTIN SAPINI
305 Old Tarrytown Road
White Plains, New York 10603
(914) 361-4888

I affirm this 9th date of April, 2026, under the penalties or perjury under the laws of New York, which may include a fine or imprisonment, that the foregoing is true, and I understand that this document may be filed in an action or proceeding in a Court of Law.



LYNN SAGL

**Emmanuel Hostin, M.D.
Board Certified Orthopaedic Surgeon
228 East 45th Street, 17th Floor
New York, NY 10017**

March 31, 2026

D&D ASSOCIATES
PO Box 9150
Garden City, NY 11530

RE:	Justin Sapini
D&D Acct#:	22726002906-1
Claim No:	SIR068120
Date of Examination:	March 31, 2026
Type of Examination:	Rescheduled Liability
Specialty:	Orthopaedic Surgeon
Date of Incident:	April 23, 2023
Carrier#:	Claims Service Bureau

To Whom It May Concern:

The claimant comes in today and presents a New York State photo ID driver's license. He is 30 years old, 5'11" tall, weighs 195 pounds, and is right-hand dominant. The claimant was seen for an independent medical examination in New York.

HISTORY OF PRESENT CONDITION:

The claimant was involved in a motor vehicle accident as a belted driver on April 23, 2023. He states that his vehicle struck another vehicle as both were merging into adjacent lanes. He describes the incident as a sideswipe collision. He notes injuries isolated to both shoulders, both knees, his left ankle, as well as his neck, upper back, and lower back.

TREATMENT FOLLOWING INJURY:

The claimant went home after the accident. He presented the following day to an emergency room. There he complained of neck, upper back and left shoulder pain as well as lightheadedness. He was evaluated, treated, and subsequently discharged. He sought additional medical care and was initiated on a course of physical therapy, which he currently attends once per week. He also receives chiropractic care once per week. He denies having received any injections. He underwent a left shoulder arthroscopy on July 26, 2023, followed by extensive postoperative physical therapy. He reports that he is scheduled for a right shoulder arthroscopy in the near future. He currently takes tramadol and ibuprofen for pain.

PRESENT COMPLAINTS:

The claimant complains of bilateral shoulder pain with overhead reaching and reaching laterally. Regarding his low back, he reports pain that radiates upward toward his neck when sitting and

Page No: 2
RE: Justin Sapini
D&D Acct #: 22726002906-1

pain that radiates downward from the neck to the lower back when standing. He states that his knees are generally stable but become painful with prolonged walking. Regarding his activities of daily living, he is able to dress himself for the most part but requires some assistance when putting on his shirt. He is able to feed and wash himself without difficulty. He is able to drive and climb stairs, although with some knee pain.

GENERAL MEDICAL HISTORY:

Past medical history – REFUSED TO PROVIDE.

Medications – Tramadol, Ibuprofen

Past surgical history – Left shoulder arthroscopy (July 26, 2023).

ACCIDENT HISTORY:

The claimant denies any prior motor vehicle accidents or prior work-related injuries and denies any subsequent injuries since the April 23, 2023 accident.

OCCUPATIONAL HISTORY:

The claimant is not currently working and has remained out of work since the date of injury. He reports prior work experience in stocking, restaurant service, and door-to-door financial advising.

REVIEW OF MEDICAL RECORDS:

- Verified bill of particulars, Index No.160000/2024.
- Legal document.
- Police accident report, dated 4/23/2023,
- Operative report of the left shoulder examination under anesthesia, left shoulder arthroscopy, posterior labral repair utilizing a 2.99mm Bio Composite Push Lock anchor and a cinch stitch, complete synovectomy, lysis of adhesions, subacromial decompression, extensive debridement of the biceps tendon, partial rotator cuff tear, and cartilage defect dated 7/26/2023, David Capiola, M.D.
- X-ray report of the left ankle dated 10/5/2011, Kenneth Blumberg, M.D. Impression: Small dorsal avulsion fracture of the anterior talus.
- X-ray report of the left foot dated 10/5/2011, Kenneth Blumberg, M.D. Impression: Small dorsal avulsion fracture of the anterior talus.
- X-ray report of the chest dated 9/10/2018, Jonathan B. Schwartz, M.D. Impression: No acute cardiopulmonary disease.
- MRI report of the cervical spine dated 5/8/2023, Jonathan B. Schwartz, M.D. Impression: Straightening of the lordosis. A shallow bulge at C5-6 flattens the left ventral margin of the cervical canal.
- MRI report of the lumbar spine dated 5/8/2023, Jonathan B. Schwartz, M.D. Impression: L5-S1: Disc bulge with lateral predominance encroaches into and narrows the lateral recesses and inferior foramina bilaterally
- MRI report of the right shoulder dated 5/24/2023, Narayan B. Paruchuri, M.D. Impression: Intermediate grade interstitial tear of the anterior half of the infraspinatus tendon, Partial tear of

Page No: 3

RE: Justin Sapini

D&D Acct #: 22726002906-1

the posterior inferior labrum at the 8 o'clock position.

- MRI report of the left shoulder dated 5/24/2023, Narayan B. Paruchuri, M.D. Impression: Partial tear of the posterior inferior labrum at the 8 to 9 o'clock position. Low-grade interstitial tear of the posterior fibers of the infraspinatus tendon. Hypoplasia of the posterior inferior glenoid reflecting posterior glenoid rim deficiency. Thickening of the inferior glenohumeral ligament which can be seen with adhesive capsulitis or scarring from a partial tear.
- MRI report of the left knee dated 7/3/2023, Narayan B. Paruchuri, M.D. Impression: Horizontal undersurface tear of the posterior horn of the medial meniscus. Prepatellar soft tissue edema reflecting soft tissue contusion.
- MRI report of the right knee dated 7/3/2023, Narayan B. Paruchuri, M.D. Impression: Horizontal undersurface tear of the posterior horn of the medial meniscus.
- 3D volumetric analysis report of the brain dated 8/8/2023, Darren M. Buono, M.D. Impression: There are 3 segments of the brain involving the orbital frontal lobes, which may reflect mild focal posttraumatic neuronal loss. The possibility of an anatomic variant or statistical anomaly is felt to be less likely. Several segments, probably in the temporal lobes, demonstrate abnormal increased brain volume, which can also be seen in the setting of traumatic brain injury. Clinical correlation with the patient's history, physical, and symptoms is recommended.
- MRI report of the brain dated 8/8/2023, Darren M. Buono, M.D. Impression: Abnormal diffusion tensor imaging. Overall, 2 out of the 12 regions of interest and 1 out of the 6 white matter tracts demonstrate abnormality. These findings are statistically significant and within a 95% probability of white matter dysfunction, which can be seen in the setting of traumatic axonal shearing injury. I do not see any other etiology to explain the decreased fractional anisotropy values. No evidence of stroke. No evidence of demyelinating disease. No structural or congenital abnormalities. However, clinical correlation is recommended. Fractional anisotropy asymmetry index: There is asymmetry in the anterior limb of the internal capsules, with the left measuring 20.32% lower than the right. Asymmetry in the genu of the internal capsule, with the left measuring 22.58% lower than the right. These findings can be seen in the setting of asymmetric axonal shearing injury utilizing the contralateral brain as an internal standard. No acute intracranial hemorrhage. However, abnormal susceptibility weighted imaging measuring with a focus of hemosiderin in the subcortical right parietal lobe at the gray-white matter junction is most compatible with sequelae of posttraumatic microhemorrhage and axonal shearing injury. There are a few subcortical right frontal lobe white matter signal foci present, more pronounced at the gray-white matter junction, in a pattern and distribution which can be seen in the setting of posttraumatic gliosis. Clinical correlation is recommended to exclude hypertensive/small vessel ischemic changes or vasospastic phenomenon associated with chronic migraine headaches. No cerebral volume loss. No structural bodies are seen. No evidence of prior stroke. No masses. No congenital brain abnormalities. Based on all the imaging findings as outlined above, the Positive Predictive Value is the probability that a patient with a positive (abnormal) test result actually has a traumatic brain injury: $\geq 95\%$. Clinical correlation with the patient's history, physical, and symptoms is recommended.
- Independent orthopedic evaluation report dated 10/31/2025, Kenneth R. Alleyne, M.D.
- Independent orthopedic evaluation report dated 9/13/2023, Richard Semble, M.D.
- Montefiore Nyack Hospital Emergency Department Records report dated 4/24/2023.
- CT scan report of the cervical spine dated 4/24/2023, Daniel Macheleder, M.D. Impression: No evidence of acute displaced fracture. Straightening of cervical lordosis may be on the basis of

Page No: 4

RE: Justin Sapini

D&D Acct #: 22726002906-1

positioning versus pain and spasm.

- CT scan report of the head dated 4/24/2023, Daniel Macheleder, M.D. Impression: No acute intracranial hemorrhage, mass effect or midline shift.
- Chiropractic initial examination narrative report dated 4/26/2023, Kevin M. Haggerty, D.C.
- Chiropractic SOAP notes dated 4/28/2023, 5/2/2023, 5/3/2023, 5/5/2023, 5/8/2023, 5/9/2023, 5/10/2023, 5/15/2023, 5/16/2023, 5/17/2023, 5/22/2023, 5/23/2023, 5/30/2023, 6/2/2023, 6/5/2023, 6/7/2023, 6/9/2023, 6/12/2023, 6/13/2023, 6/14/2023, 6/23/2023, 6/27/2023, 6/28/2023, 6/30/2023, 7/3/2023, 7/5/2023, 7/7/2023, 7/10/2023, 7/12/2023, 7/21/2023, 7/24/2023, 7/31/2023, 8/7/2023, 8/8/2023, 8/9/2023, 8/14/2023, 8/16/2023, 8/21/2023, 8/22/2023, 8/23/2023, 8/29/2023, 8/30/2023, 9/11/2023, 9/12/2023, 9/19/2023, 9/20/2023, 9/27/2023, 10/10/2023, 10/13/2023, 10/17/2023, 11/7/2023, 11/21/2023, Advanced Rockland Chiropractic Offices, P.C.
- Work status reports dated 5/9/2023, 5/23/2023, 6/7/2023, 6/23/2023, 7/21/2023, 8/9/2023, 8/30/2023, 9/12/2023, 10/10/2023, 11/21/2023, Kevin M. Haggerty, D.C.
- Prescription and letter of medical necessity for medical equipment dated 5/22/2023, Kevin M. Haggerty, D.C.
- Chiropractic re-examination narrative reports dated 6/27/2023, 8/30/2023, Kevin M. Haggerty, D.C.
- EMG/NCV study report of the upper extremity dated 8/23/2023, Melinda L. Miller, M.D. Impression: The above electrodiagnostic study reveals evidence of a left C5-C6 radiculopathy. The NCV study revealed a peripheral neuropathy affecting the sensory radial nerve on the left.
- EMG/NCV study report of the lower extremity dated 9/20/2023, Melinda L. Miller, M.D. Impression: The above electrodiagnostic study reveals evidence of bilateral L5-S1 radiculopathy. There is no evidence of peripheral neuropathy at this time.
- Disability evaluation report dated 12/6/2023, David Capiola, M.D.
- Physiatrist initial evaluation report dated 5/4/2023, Vadim Abramov, M.D.
- Postoperative note dated 9/7/2023, Vadim Abramov, M.D.
- Initial examination report dated 5/17/2023, David Capiola, M.D.
- Referral forms dated 6/7/2023, 7/5/2023, 8/16/2023, 9/20/2023, David Capiola, M.D.
- Follow-up reports dated 7/5/2023, 8/16/2023, 9/20/2023, 10/18/2023, 12/6/2023, 1/3/2024, David Capiola, M.D., Nicole Kosuda, P.A.
- Physical therapy initial examination report dated 6/29/2023, Michael Ludena, P.T.
- Physical therapy daily notes dated 6/29/2023, 7/6/2023, 7/11/2023, 7/13/2023, 7/20/2023, 9/14/2023, 9/16/2023, 9/21/2023, 9/26/2023, 10/6/2023, 10/11/2023, 10/16/2023, 10/23/2023, 10/25/2023, 11/1/2023, 11/8/2023, 12/4/2023, 12/7/2023, 12/11/2023, 12/14/2023, Michael Ludena, P.T.
- Physical therapy discharge note dated 9/14/2023, Michael Ludena, P.T.
- Physical therapy re-examination report dated 9/14/2023, Michael Ludena, P.T.
- Comprehensive neurological examination reports dated 5/31/2023, 9/6/2023, Peter C. Kwan, M.D.
- Neurological follow-up evaluation reports dated 10/31/2023, 12/12/2023, Peter C. Kwan, M.D.
- MRI report of the right knee dated 7/3/2023, Marc J. Katzman, M.D. Impression: No recent traumatic injury or internal derangement.
- MRI report of the left knee dated 7/3/2023, Marc J. Katzman, M.D. Impression: No recent

Page No: 5

RE: Justin Sapini

D&D Acct #: 22726002906-1

traumatic injury or internal derangement.

- MRI report of the right shoulder dated 5/24/2023, Marc J. Katzman, M.D. Impression: No evidence of recent traumatic injury to the right shoulder. The examination is remarkable.
- X-ray report of the chest dated 9/10/2018, Marc J. Katzman, M.D. Impression: The chest X-ray reveals no acute chest findings.
- MRI report of the brain dated 8/8/2023, Marc J. Katzman, M.D. Impression: No evidence of traumatic injury to the brain. There is no intracranial haemorrhage, extra-axial hematoma, cerebral contusion, or posttraumatic encephalomalacia. The examination is normal.
- DTI report of the brain dated 8/8/2023, Marc J. Katzman, M.D. Impression: The DTI study of the brain performed on 08/08/2023 is within normal limits and reveals no evidence of significant white matter microstructural disruption.
- MR neuroquant report of the brain dated 8/8/2023, Marc J. Katzman, M.D. Impression: The neuroquant volumetric analysis of the brain performed on 08/08/2023 reveals no evidence of age-disproportionate or post-traumatic brain atrophy.

PHYSICAL EXAMINATION:

The claimant has a normal gait and is not using any assistive device. He is able to get on and off the examination table without difficulty. Notably, during the interview portion of the examination, he was seated upright with good posture and did not lean against the chair.

The ranges of motion of the examined body parts were performed by the claimant. This is a subjective maneuver on the part of the claimant. The measurement itself is an objective measurement of the claimant's subjective efforts. Ranges of motion are taken both visually and with the use of a handheld goniometer. Normal ranges of motion are according to the AMA Guides to the Evaluation of Permanent Impairment, 5th Edition, and Measurement of Joint Motion, A Guide to Goniometry, 3rd Edition. Variations occur according to age and sex.

CERVICAL SPINE:

There is a negative Spurling sign. There is no paravertebral tenderness or muscle spasm. There is a normal cervical lordosis. There is intact strength, reflex and sensation in the upper extremities. There is no atrophy.

Range of Motion	Normal	Measured
Flexion	40-50	40
Extension	50-60	50
Right rotation	80	80
Left rotation	80	80
Right lateral bending	20-45	30
Left lateral bending	20-45	30

THORACIC SPINE:

Non-tender to paraspinal palpation and no paraspinal muscular spasm.

Page No: 6

RE: Justin Sapini

D&D Acct #: 22726002906-1

Range of Motion (in degrees)	Normal	Measured
Flexion	20 to 45	20
Extension	25 to 45	20
Right rotation	35 to 50	refused
Left rotation	35 to 50	refused
Right lateral bending	20 to 40	20
Left lateral bending	20 to 40	20

LUMBAR SPINE:

There was no spine pain to skull compression. There was positive pain in the lower back with passive rotation of the torso while maintaining alignment between the hips and the shoulders. Normal lumbar lordosis. There was mild paravertebral tenderness with no muscle spasm. There was a negative straight leg raise test bilaterally. There were intact motor strength, reflex, and sensation in the lower extremities. There was no atrophy of the lower extremities.

Range of Motion	Normal	Measured
Flexion	60	30
Extension	25	15
Right rotation	80	refused
Left rotation	80	refused
Right lateral bending	25	20
Left lateral bending	25	20

BILATERAL SHOULDER:

There is guarding with attempts at passive range of motion.
Hawkin's impingement sign is negative bilaterally.
There is no AC joint or bicipital groove tenderness.
Rotator cuff tear 5/5 bilaterally.

Range of Motion	Normal	Measured Right	Measured Left
Flexion	180	130	120
Abduction	180	120	115
External Rotation in adduction	60	50	50
External Rotation in abduction	90	80	80
Internal Rotation (to Spinous Process level)	varied	L2	L4
Posterior extension	60	60	60
Adduction	30	30	30

Page No: 7

RE: Justin Sapini

D&D Acct #: 22726002906-1

BILATERAL ELBOW:

No tenderness over the medial or lateral epicondyle or olecranon tip. Tinel's sign is positive over the left cubital tunnel and negative on the right.

Range of Motion	Normal	Measured Right	Measured Left
Flexion	150	150	150
Extension	0	0	0
Pronation	90	90	90
Supination	90	90	90

BILATERAL WRIST:

No tenderness over the radial or ulnar styloid. Finkelstein's test and Tinel's sign at the carpal tunnel are negative.

Range of Motion (in degrees)	Normal	Measured Right	Measured Left
Dorsiflexion	70	70	70
Palmar flexion	80	80	80
Pronation	90	90	90
Supination	90	90	90
Radial deviation	20	20	20
Ulnar deviation	30	30	30

BILATERAL HAND:**BILATERAL THUMB:**

Range of Motion (in degrees)	Normal	Measured Right	Measured Left
Interphalangeal joint	0-80	0-80	0-80
Metacarpophalangeal joint	0-60	0-60	0-60
Carpometacarpal flexion	0-60	0-60	0-60
Radial Abduction	0-90	0-90	0-90
Opposition	To which finger MCP	To which finger MCP	To which finger MCP
Isolated Opposition	Test to each digit	Test to each digit	Test to each digit

Page No: 8
RE: Justin Sapini
D&D Acct #: 22726002906-1

FINGERS:

Range of Motion (in degrees)	Normal	Measured Right	Measured Left
DIP Jt (distal IP Jt)	0-90	0-90	0-90
PIP Jt (proximal IP Jt)	0-100	0-100	0-100
MCP Jt (Metacarpophalangeal Jt)	0-90	0-90	0-90

BILATERAL KNEE:

No joint line tenderness, effusion, or ligamentous instability. The patellofemoral compression test is negative bilaterally.

Range of Motion (in degrees)	Normal	Measured Right	Measured Left
Flexion	140	110	125
Extension	0	0	0
Hyperextension	0-5	0	0

DIAGNOSIS:

1. Neck sprain – resolved
2. Mid back sprain – resolved
3. Low back sprain – resolved
4. Bilateral shoulder sprain -- resolved
5. Bilateral elbow sprain – resolved
6. Bilateral knee sprain – resolved
7. Bilateral wrist sprain – resolved

OPINION:

- After taking a complete history, reviewing the provided records, and performing a physical examination, with a reasonable degree of medical certainty, the claimant's condition has plateaued and is medically stationary.
- The claimant presented with subjective complaints which did not correlate with the radiographic findings or other objective findings. The measured limitations in lumbar spinal range of motion were observed and measured in response to my direct request for demonstration, however, at other points in the encounter (including transfers and ambulation), the claimant's lumbar spinal range of motion and spinal dynamics revealed no evidence of dysfunction or restriction. Specifically, he sat with firm posture during the

Page No: 9

RE: Justin Sapini

D&D Acct #: 22726002906-1

interview portion of the exam and did not require the back support of the chair in which he sat. The demonstrated range of motion deficits were found to be unassociated with objective evidence of pathology including a lack of spasm or contracture.

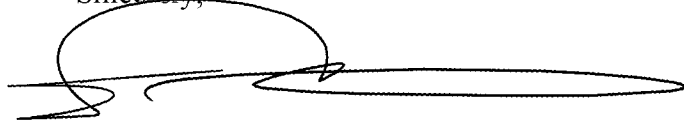
- The range of motion findings of the bilateral shoulders and bilateral knees were unassociated with objective evidence of pathology, including no evidence of spasm, contracture, or bony derangement. These deficits were related to the claimant's subjective concerns regarding the potential provocation of pain. Significant guarding against passive range of motion was noted with regard to the shoulders and his knees.
- The positive Tinel's over the left cubital tunnel indicates an increased irritability of the ulnar nerve at that site. The claimant had no complaints consistent with ulnar neuritis and as such, no treatment for this finding is indicated.
- The claimant is able to return to work without restrictions.

Thank you for allowing me to evaluate Mr. Justin Sapini. If you have further questions, please do not hesitate to contact me.

Pursuant to CPLR 2106, I affirm this 31st day of March 2026 under the penalties of perjury under the laws of New York, which may include a fine or imprisonment, that the foregoing is true, except as to matters alleged on information and belief and as to those matters I believe it to be true, and I understand that this document may be filed in an action or proceeding in a court of law.

I affirm to having a scope of licensure or certification that typically manages the medical condition, procedure, treatment, or issue in this case. I have current, relevant knowledge and experience to render an opinion for this case, and my opinions and conclusions are based solely upon the review of the records submitted as well as the results of my examination and/or review. There is no conflict of interest known to me regarding the specific case. I have received no financial incentive or compensation that is dependent in any way on the opinion I have rendered. No delegation of this examination and/or review was rendered.

Sincerely,

A handwritten signature in black ink, appearing to read 'Emmanuel Hostin', with a large, sweeping horizontal stroke underneath.

Emmanuel Hostin, M.D.
Orthopaedic Surgeon

EMMANUEL HOSTIN, M.D., F.A.A.O.S.

Employment / Hospital Affiliations**EMPLOYMENT****SOLO PRACTICE:**

FEB 2006 TO PRESENT

HOSTIN ORTHOPAEDICS & SPORTS MEDICINE, PC**HOSPITAL / AMBULATORY CARE CENTER AFFILIATIONS**

MOUNT SINAI WEST HOSPITAL (NYC, NY)	2/2016 TO
PRESENT	
LENOX HILL HOSPITAL (NYC, NY)	10/2002 TO
2018	
MANHATTAN EYE, EAR AND THROAT HOSPITAL (NYC, NY)	10/2002 TO
2018	
EMPIRE STATE AMBULATORY SURGICAL CENTER (BRONX, NY)	5/2014 TO
PRESENT	
MANHATTAN SURGICAL CENTER (ROOSEVELT HOSP) (NYC, NY)	9/2015 TO
PRESENT	
BOULEVARD SURGICAL CENTER (LONG ISLAND CITY, NY)	1/2006 TO 2018
MOUNT SINAI OF QUEENS HOSPITAL (ASTORIA, NY)	11/2008 TO
12/2013	
NEW YORK HOSPITAL OF QUEENS (FLUSHING, NY)	12/2007 TO
12/2010	
SURGICARE AMBULATORY SURGICAL CENTER OF NY (BRONX, NY)	7/2007 TO
5/2015	
HILLSIDE SURGICAL CENTER (HOLLIS, NY)	7/2010 TO
1/2012	

Education / Postgraduate Training**SPORTS MEDICINE FELLOWSHIP:****BOOTH BARTOLOZZI BALDERSTON ORTHOPAEDICS/ PENN
ORTHOPAEDICS UNIVERSITY OF PENNSYLVANIA HEALTH
SYSTEM**

Philadelphia, PA

*August 2001 to July 2002***ORTHOPAEDIC SURGERY RESIDENCY:****JOHNS HOPKINS MEDICAL INSTITUTION**

Baltimore, MD

*July 1996 to June 2001***MEDICAL SCHOOL:****JOHNS HOPKINS UNIVERSITY SCHOOL OF MEDICINE**

Baltimore, MD

Sept 1992 to May 1996 (M.D. degree conferred)

UNDERGRADUATE DEGREE:**THE JOHNS HOPKINS UNIVERSITY**

Baltimore, MD

Sept 1988 to May 1992 (B.A., Chemistry, Summa Cum Laude)

- Phi Beta Kappa 1992

HIGH SCHOOL:**CHAMINADE HIGH SCHOOL**

Mineola, NY

Sept 1984 to June 1988

- Chaminade Man Award (Highest honor awarded)
- Salutatorian

Licensure / Certification

NEW YORK STATE (LICENSE # 60 225878)

AUGUST 2002 (CURRENT)

AMERICAN BOARD OF ORTHOPAEDIC SURGERY

BOARD CERTIFIED JULY 2005

FELLOW OF THE AMERICAN ACADEMY OF ORTHOPAEDIC SURGERY

Personal Data

PLACE OF BIRTH:

MADRID, SPAIN (FLUENT IN SPANISH)

CITIZENSHIP:

UNITED STATES (NATIVE OF NEW YORK)

MARITAL STATUS:

MARRIED

Special Interests

- OPEN AND ARTHROSCOPIC SHOULDER SURGERY
- ARTHROSCOPIC SURGERY OF SHOULDER, ELBOW, KNEE, ANKLE
- KNEE, HIP & SHOULDER REPLACEMENT

Publications

- 1 Hostin E, Mont MA, Mayerson JA, Jones LC, Hungerford DS: Total hip arthroplasty in patients receiving Workers' Compensation. *Clin Orthop*. 2000 Oct (379):161-8.
- 2 M.A. Mont, K. Low, D.M. LaPorte, E Hostin, L.C. Jones, & D.S. Hungerford: Reinfusion Drains After Primary Total Hip & Total Knee Arthroplasty. *J So Orth Assn*, Fall 2000.
- 3 Waldman BJ, Hostin E, Mont MA, Hungerford DS. Infected total knee arthroplasty treated by arthroscopic irrigation and debridement. *J Arthroplasty*. 2000 Jun;15(4):430-6.
- 4 Domb B, Hostin E, Mont MA, Hungerford DS: Cortical strut grafting for enigmatic thigh pain following total hip arthroplasty. *Orthopedics* 2000 Jan;23(1) 21-4
- 5 E. Hostin, I.H. Pacheco, M. Urquhart, L.C. Jones, M.A. Mont, M.A. Jacobs: The Need for Reinfusion Drains after Primary Total Hip and Total Knee Arthroplasty. Abstract. *Orthop. Trans.* 20(4): 895, 1996-1997

6. B.I. Dahiyat, E. Hostin, E.M. Posadas, K.W. Leong: Synthesis and characterization of putrescine-based poly(phosphoester-urethanes). *Journal of Biomaterials Science*, Vol.4, No.5, pp.529-543 (1993)